

Trial	RESCUE-Japan LIMIT (N=203)	SELECT2 (N=352)	ANGEL-ASPECT (N=456)
Population - LNW ≤24h - Prestroke mRS 0-1	≥18y - Cervical/Intracranial ICA or M1 segment MCA occlusion - ASPECTS 3-5 (CT/MRI) - LNW 6-24h → negative FLAIR MRI - NIHSS ≥6 - Japan	18-85y - Cervical/intracranial ICA or M1 segment - ASPECTS 3-5 (CT) or CTP/MRI core ≥ 50cc - North America, Europe, Australia/NZ	18-80y - Intracranial ICA or M1 segment MCA
Intervention / Control	1:1 EVT + medical tx vs. medical tx alone *IV thrombolytics, if eligible (RESCUE-Japan LIMIT: IV-tPA 0.6 mg/kg; SELECT2: alteplase/TNK; ANGEL-ASPECT: alteplase/urokinase)		
1° Outcome (3-month)	% mRS 0-3	Ordinal shift mRS	Ordinal shift mRS
Baseline Characteristics: - Mean/Median Age - Median NIHSS - Median ASPECTS - Time (LNW→Rand) - IV thrombolytic - TICI ≥2b achieved	76y 22 3 3.7h 27% 86%	66.5y 19 4 (mean CTP core 80cc) 9.3h 19% 80%	68y 16 3 7.6h 28% 81%
Tx Effect (ITT) *as-treated analyses → similar results	EVT: 31%, Medical: 12.7%; relative risk 2.43 (1.35-4.37)	EVT: 4, Medical: 5; generalized odds ratio 1.51 (1.20-1.89)	EVT: 4, Medical: 4; generalized odds ratio 1.37 (1.11-1.69)
2° Efficacy Outcomes	- mRS 0-2 / 0-1: no diff - Ordinal shift mRS: cOR 2.4 (1.5-4.0) - Early neurologic improvement (↓ of 48h NIHSS ≥8): RR 3.5 (1.8-7.0)	- mRS 0-2: 3.0 (1.6-5.5) - mRS 0-3: 2.1 (1.4-3.0) - Early neurologic improvement (↓ of 24h NIHSS ≥8): no diff	- mRS 0-2: RR 2.6 (1.6-4.1) - mRS 0-3: RR 1.5 (1.2-1.9) - Early neurologic improvement (↓ of 36h NIHSS ≥10 or NIHSS 0-1): 4.3 (1.3-14.5)
Safety Outcomes	- No diff in Sx ICH, early death, decompressive crani - Any ICH: 1.9 (1.3-2.6)	- No diff in Sx ICH or PH2, 3m death - Early neurologic worsening (↑ in 24h NIHSS ≥4): 1.6 (1.0-2.5)	- No diff in Sx ICH, 3m death, decompressive crani - Any ICH: 2.7 (1.9-3.8)
Limitations	- ASPECTS mainly estimated w/ MRI - Excluded + MRI-FLAIR if LNW≥6h 0.6 mg/kg IV-tPA - Restricted to Japanese population	Terminated early (interim analysis RESCUE-Japan LIMIT → target N=560)	- Terminated early (prespecified interim analysis → superiority, target N=502) - IV thrombolysis w/ urokinase in minority - Mainly Han Chinese population

Source-qualified teaching update | 30 September 2026

ANGEL-ASPECT protocol: NIHSS 6-30; ASPECTS 3-5 without a core-volume restriction; ASPECTS <3 with core 70-100 mL within 24 hours; or ASPECTS >5 with core 70-100 mL at 6-24 hours. These trial criteria are distinct from current EVT guidance.

[pmc.ncbi.nlm.nih.gov/articles/PMC10176983/](https://pubmed.ncbi.nlm.nih.gov/articles/PMC10176983/)
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SELECT2 correction: the proportion meeting both ASPECTS ≤5 and core ≥50 mL was 66.8%, not 78%. This source correction does not change the direction or significance of the main treatment result.

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Subgroup Analyses	Treatment effect similar across: • age (75y) • time LNW→rand (6h) • NIHSS (21) • IV-tPA tx	Treatment effect similar across: • age (70y) • NIHSS (20) • ICA/MCA occlusion site • CTP core estimates (70/100/150cc) • ASPECTS (0-2/3-5/6-10) • mismatch ratio/volume (1.8/15cc, 1.2/10cc) • affected hemisphere	Treatment effect similar across: • age (75y) • wake-up stroke • time LNW→rand (6h) • NIHSS (16) • thrombolytic tx • occlusion site (ICA/MCA) • ipsilateral cervical ICA occlusion • ASPECTS (<3/≥3) • infarct core (70cc) • ischemic stroke subtype
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* majority at least moderately disabled, <50% able to ambulate independently, and 20-40% dead @3m

* any ICH common, but sx ICH rare